This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of meningococcal group B vaccine (Bexsero and Trumenba)

for protection against invasive meningococcal group B disease

Patient Group Direction, version 008, issued 30 September 2026. Supersedes Version 007, 14 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Guideline Summary: Green Book chapter 22 (Meningococcal)

The disease

- Invasive meningococcal disease is fatal in 5% to 10% of cases, and survivors may be left with hearing loss, severe visual impairment, limb amputation, seizures or brain damage.
- Incidence is highest in infants under one year, with a second peak in 15 to 19 year olds. Living in closed or semi-closed communities such as university halls raises the risk.
- Group B has become re-established across all age groups since 2021/22, while disease from the other groups remains very rare.

The two vaccines are not interchangeable in schedule

- Bexsero is the four-component protein vaccine (4CMenB), licensed from 2 months of age, and is the vaccine used in the routine infant programme.
- Trumenba is the bivalent fHbp vaccine (MenB-fHbp), authorised for individuals aged 10 years and over as a 2 or 3 dose schedule. It has no role below 10 years.
- They have different antigens, different licensed ages and different schedules. Choose the product first, then follow that product’s schedule; do not mix schedules between them.
- Where a course has been started with one product, complete it with the same product wherever possible.

Where a pharmacy service fits

- The routine infant doses at 8 weeks, 12 weeks and 12 months are given by the NHS programme. Families should be directed there rather than paying privately.
- This PGD is for privately funded vaccination: infants and children whose routine doses were missed or who present outside the programme, adolescents and students seeking protection, and adults at increased risk.
- MenB vaccine is NOT recommended for travel. Travellers needing meningococcal protection require MenACWY; use the MenACWY PGD, including for Hajj and Umrah where a certificate is a visa requirement.
- Vaccines for private, occupational health or travel use are not supplied free of charge and must be ordered from the manufacturer.

Groups at increased risk

- Asplenia or splenic dysfunction, complement disorders, and treatment with a complement inhibitor such as eculizumab all raise the risk of invasive meningococcal disease substantially.
- Individuals due to start a complement inhibitor should be vaccinated at least 2 weeks before treatment begins, unless the risk of delaying treatment outweighs the risk of infection. Where treatment starts less than 2 weeks after vaccination, prophylactic antibiotics are required until 2 weeks after the vaccine.

- Laboratory staff handling Neisseria meningitidis require MenACWY (one dose) and 4CMenB (two doses), with boosters of both every 5 years.
- The need for, and timing of, booster doses of MenB vaccine in at-risk individuals has not yet been determined.

Patient Group Direction

for the administration of

Meningococcal group B vaccine: Bexsero (4CMenB) and Trumenba (MenB-fHbp)

for protection against invasive meningococcal group B disease

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must have completed immunisation training in line with the National Minimum Standards and Core Curriculum for Immunisation Training ● All users must be competent in the recognition and management of anaphylaxis, and in vaccine handling and cold chain management ● All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required ● All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so. ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005 ● All users must be familiar with Gillick competence and the assessment of consent in children and young people, and with the involvement of a person with parental responsibility

Clinical condition or situation to which this PGD applies

PGD Indication	Active immunisation against invasive disease caused by Neisseria meningitidis group B, in accordance with Chapter 22 of Immunisation Against Infectious Disease (the Green Book) and the current SPC for the product used. Bexsero (4CMenB) may be given from 2 months of age; Trumenba (MenB-fHbp) from 10 years of age. This PGD covers privately funded vaccination and does not replace the NHS routine infant programme.
Inclusion criteria	● Aged 2 months and over for Bexsero, or 10 years and over for Trumenba. ● Requires protection against meningococcal group B disease: routine doses missed or presenting outside the NHS programme, an adolescent or student seeking protection, or an adult at increased risk (asplenia or splenic dysfunction, a complement disorder, complement inhibitor therapy, or laboratory work with Neisseria meningitidis). ● Valid informed consent obtained, from a person with parental responsibility where the individual is under 16 and not Gillick competent. ● A person with parental responsibility, or a suitable adult authorised by them, is present for the vaccination of a child under 16 years. ● No contraindication as set out below.
Exclusion criteria	● Aged under 2 months. ● Aged under 10 years where Trumenba is the only product available. Trumenba has no licence below 10 years; use Bexsero or refer. ● Confirmed anaphylactic reaction to a previous dose of the same vaccine. ● Confirmed anaphylactic reaction to any component of the vaccine, or to any residue from the manufacturing process. ● Acute severe febrile illness. Postpone until recovered. A minor illness without fever or systemic upset is not a reason to postpone. ● Valid informed consent not obtained, or no person with parental responsibility available to consent for a child under 16 who is not Gillick competent. ● A request for MenB vaccination for travel purposes. MenB is not recommended for travel; assess for MenACWY under the relevant PGD instead. ● Management of a case, contact or outbreak of meningococcal disease. This is directed by the local Health Protection Team and is outside this PGD.
Cautions including any relevant action to be taken	● Facilities and trained staff for the management of anaphylaxis must be available, with immediate access to adrenaline (epinephrine) 1 in 1,000 injection and a telephone. ● Prophylactic paracetamol is advised where Bexsero is given to infants under one year, because fever is common, particularly when given with other routine vaccines. Give 2.5 ml of infant paracetamol 120mg/5ml as soon as possible after vaccination, a second dose after 4 to 6 hours and a third 4 to 6 hours after that. Ibuprofen is less effective and is not recommended. ● Advise the parent or carer to seek medical advice if the child is

	noticeably unwell with a fever, or if fever occurs at other times. ● Asplenia, splenic dysfunction, complement disorders and complement inhibitor therapy: vaccinate, and involve the specialist team. An individual due to start a complement inhibitor should be vaccinated at least 2 weeks before treatment begins; where treatment starts sooner, prophylactic antibiotics are required until 2 weeks after vaccination, arranged by the specialist team (see the guidance summary in part 2). ● Immunosuppression and HIV, regardless of CD4 count: vaccinate in accordance with the routine schedule, but the individual may not make a full antibody response. Re-immunisation may be considered after treatment finishes; specialist advice may be required. ● Pregnancy and breastfeeding: meningococcal vaccines may be given when clinically indicated. There is no evidence of risk from vaccinating pregnant or breastfeeding women with inactivated vaccines. ● A previous systemic or local reaction to a meningococcal vaccine, including fever of any severity, a hypotonic-hyporesponsive episode, persistent crying for more than 3 hours, a severe local reaction of any extent, or a convulsion within 3 days, does not prevent further doses. ● Very premature infants born at or before 28 weeks who are in hospital should have respiratory monitoring for 48 to 72 hours after their first immunisation. Immunisation should not be withheld or delayed. ● Where given with other vaccines, use a separate site, preferably a different limb, or at least 2.5 cm apart in the same limb. Record the site of each. ● Observation after vaccination: observe every patient for 15 minutes after vaccination, seated. Anxiety-related reactions, including vasovagal syncope, hyperventilation and transient visual disturbance or paraesthesia, can occur before or after any injection and are commonest in adolescents; they are a response to the needle, not to the vaccine. Have procedures in place to prevent injury from a faint, and record that the observation period was completed.
Actions if patient is excluded or declines treatment	Advise on alternative options and how these can be accessed, including the NHS routine programme for infants, their GP practice, and the MenACWY PGD where the request is travel related. Explain the risks of meningococcal disease and the benefit of vaccination. Document any advice given and the decision reached. Inform or refer to the GP as appropriate. Where the individual is at increased risk through asplenia, a complement disorder or complement inhibitor therapy, make the referral clear and timely.

Vaccines and schedules covered by this PGD

Green Book schedule guidance supersedes the SPC. Confirm the current schedule and the licensed age before administration.

Vaccine	Age	Dose and route	Schedule
Bexsero (4CMenB, GSK). Contains NHBA, NadA and fHbp 50 micrograms	From 2 months	0.5 ml intramuscular. Anterolateral thigh for infants aged 1 year and under, deltoid for older	Infants under 12 months outside the NHS programme: 2 doses at least 4 weeks apart, followed by a booster at 12 months. Children 12 months to under 2 years, according to the

each, plus 25 micrograms outer membrane vesicles		children and adults	Bexsero doses given in the first year (Bexsero SmPC section 4.2, Table 1): none, 2 doses at least 2 months apart, followed by a booster 12 to 23 months after the second dose; one, one further dose at least 2 months after it to complete the primary course, followed by a booster 12 to 23 months after that dose; two, a single booster dose at least 2 months after the second primary dose (at least 6 months where the primary course was given at 2 to 5 months of age) and before the second birthday. Aged 2 years and over, including adolescents and adults: 2 doses at least 1 month apart; a child whose 2 dose primary course was completed at 12 to 23 months of age has the booster 12 to 23 months after the second primary dose.
Trumenba (MenB-fHbp, Pfizer). Contains recombinant fHbp subfamily A and subfamily B, 60 micrograms each, adsorbed on aluminium phosphate	From 10 years	0.5 ml intramuscular, deltoid	Routine use: 2 doses at 0 and 6 months. Individuals at increased risk (see inclusion criteria): 3 doses at 0, 1 to 2 months, and 6 months. Outbreak management is outside this PGD.

Details of the medicines

Legal category	POM
Off-label use	This PGD does not authorise off-label use. Administration must be within the licensed age and dose for the product supplied, following the Green Book schedule where it differs from the SPC.
Storage	Store at +2°C to +8°C. Do not freeze; discard if frozen. Store in the original packaging to protect from light. Vaccine exposed to conditions outside this range must be quarantined and risk assessed in accordance with UKHSA Vaccine Incident Guidance before any decision on continued use.
Route/method of administration	● Intramuscular injection. Use the anterolateral thigh for infants aged 1 year and under, and the deltoid for older children and adults. ● Inspect visually before administration and do not use if the appearance differs from that described in the SPC. ● May be given at the same time as any other vaccine required, at a separate site.
Dose and frequency of administration	As set out in the schedule table above, according to the product used and the age of the individual. One 0.5 ml dose per administration.
Quantity to be administered and/or supplied	One 0.5 ml dose per administration. Book the next dose in the course at the same appointment.
Maximum or minimum treatment period	The course for the product and age as set out above. The need for, and timing of, further booster doses in at-risk individuals has not yet been determined, other than the 5 yearly boosters for laboratory staff handling Neisseria meningitidis.
Disposal	Dispose of used syringes, needles and any discharged vaccine in a UN-approved puncture-resistant sharps container in accordance with local arrangements and HTM 07-01.
Drug interactions	May be given at the same time as other vaccines at a separate site. The

	immune response may be reduced in individuals who are immunosuppressed; this is not a reason to withhold vaccination.
Adverse effects	Bexsero in adolescents and adults: injection site pain, malaise and headache are most common. In infants and children up to 10 years: injection site reactions, fever of 38°C or above and irritability are very common, with diarrhoea, vomiting, feeding problems, sleepiness, unusual crying and rash also common. Fever peaks at around 6 hours and has usually gone by 48 hours. Trumenba in those aged 10 and over: headache, diarrhoea, nausea, muscle pain, joint pain, fatigue, chills, and injection site pain, swelling and redness. Refer to the current SPC for the full list.
Yellow card reporting	Report suspected adverse reactions to the MHRA via the Yellow Card scheme at https://yellowcard.mhra.gov.uk. All suspected reactions in children, and serious reactions in adults, should be reported. Document any adverse reaction in the individual's record and inform their GP.
Records to be kept	Record the following information: ● that valid informed consent was given by the individual, or by a person with parental responsibility, or that a decision was made in the individual's best interests in accordance with the Mental Capacity Act 2005 ● name of individual, address, date of birth and GP with whom the individual is registered ● name of immuniser ● name and brand of vaccine ● date of administration ● dose, form and route of administration ● quantity administered ● batch number and expiry date ● anatomical site of vaccination ● advice given, including advice given if the individual is excluded or declines immunisation ● details of any adverse drug reactions and actions taken ● that the vaccine was administered via PGD ● which product was given and which dose in the course it represents, so the course can be completed with the same product Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. The individual's GP should be informed. ● Adults aged 18 years and over: keep records for 8 years ● Children aged under 18 years: keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Offer the marketing authorisation holder's patient information leaflet for the product given, and a written record showing the product, the date, and when the next dose is due. For infants receiving Bexsero, give written paracetamol advice.
Follow-up advice to be given to patient or carer	Explain that the course must be completed for full protection and confirm when the next dose is due. Counsel on expected side effects and their management, and on Yellow Card reporting. Explain that the vaccine does not protect against all causes of meningitis and septicaemia, so the

	signs of meningococcal disease still matter: seek urgent medical help for a fever with a rash that does not fade under pressure, severe headache, neck stiffness, dislike of bright light, drowsiness or confusion. For infants given Bexsero, explain the expected fever and the paracetamol schedule.

Key references

- Immunisation Against Infectious Disease (the Green Book), Chapter 22 (Meningococcal), 10 June 2025
- Summary of Product Characteristics for Bexsero (GSK) and Trumenba (Pfizer), current versions
- UKHSA: Meningococcal disease, guidance on public health management
- NICE NG240: Meningitis (bacterial) and meningococcal disease
- UKHSA Vaccine Incident Guidance
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates	

Name of healthcare professional	Job title	Registration number	Signature

Valid from date	Expiry date
30/9/26	31/7/27

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development and issue of new PGD	30th July 2026

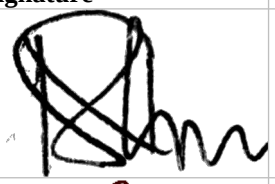
Version and change record
Version: v008
Issued: 30 September 2026
Supersedes: Version 007, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v007
Issued: 14 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision 9: the Bexsero schedule for children aged 12 months to under 2 years was rewritten to the Bexsero SmPC (section 4.2, Table 1, revised 14 March 2025): an unvaccinated child has 2 doses at least 2 months apart followed by a booster 12 to 23 months after the second dose; a child with one dose in the first year completes the primary course with one further dose at least 2 months later and then has the same booster; a child with two doses in the first year has a single booster before the second birthday; and the 2 years and over row now carries the booster for a primary course completed at 12 to 23 months. The consultation tool's doses-in-first-year field drives the same schedule. The one-dose-in-the-first-year arm interpolates between the SmPC rows and is for Chris Pilkington to confirm.
Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.
Previous version records
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
The inclusion criterion now names the increased risk groups (asplenia or splenic dysfunction, complement disorders, complement inhibitor therapy, laboratory work with Neisseria meningitidis) that the indication and the guidance summary describe.
The complement inhibitor caution now states the timing requirement itself instead of referring to 'the timing requirements above', which were in part 2.

The Bexsero schedule table and the route row now give the same injection site rule: anterolateral thigh for infants aged 1 year and under, deltoid for older children and adults.
The Trumenba 3 dose schedule no longer mentions an outbreak setting, which is an exclusion under this PGD.
The duplicate 'batch number, expiry date and anatomical site' line in the records row was removed; both items remain in the list.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v004
Issued: 11 September 2026
Supersedes: Version 003, 9 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Rebuilt from the version 002 source. The 15-minute observation requirement that version 003 carried as an appended addendum page is now written into the cautions, where it belongs; the addendum page, its narrative and the second signature page are gone. Versions 001 to 003 are listed in the change history.
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Signed on behalf of Get Real Health

Version v008, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.